



P12 · 28 PAGES, WITH A DECISION TREE

A Breastfeeding Troubleshooting Guide

A 28-page printable guide with a latch checklist, a quick-reference position library, and a what-feels-off decision tree. Built to bridge the gap between the lactation visit and the next one. Not medical advice. Made to hand to a real provider.

Soothemade Notes

A SMALL APOTHECARY OF PRINTABLES · MADE SLOWLY
A PLANNING AND TRACKING TOOL · NOT MEDICAL ADVICE

Before you open this

If you have any of the following, do not look it up in this guide. Call your provider or the breastfeeding helpline today.

- A fever above 100.4°F with body aches
- A red, hot, painful, hardening section of breast that does not soften with feeding or massage
- Cracks or bleeding that have not improved in 48 hours of trying basic fixes
- Your baby is not having wet diapers, is hard to wake for feeds, or is losing weight at the doctor's visit
- You are in pain so consistent that you are dreading feeds

The US National Breastfeeding Helpline is **1-800-994-9662**, Monday to Friday, free, in English and Spanish.

This guide is a reference. It is not a diagnosis tool. A trained lactation consultant (IBCLC) is the right person to actually see what is happening. Use this guide to organize what you've noticed, then bring it to them.

Soothemade

A note from me

The first time my milk came in, I sat on the couch crying because I could not figure out why it hurt. The nurse at the hospital had said it would be “uncomfortable for the first few days.” It was not uncomfortable. It felt like a stapler at the wrong angle each time the baby latched. I genuinely thought something was permanently wrong with me.

A lactation consultant came over on day five, watched one feed for forty-five seconds, and said: “tip his head back a little more.” That was it. That was the whole thing.

Most early breastfeeding pain is fixable with one small adjustment. The hard part is figuring out which adjustment, because you cannot see your own latch, and the baby cannot tell you what’s wrong, and the internet at 3am is a thicket.

This guide is the version of that lactation consultant you can keep on the nursing chair. It will not replace her. But it will help you ask better questions when you see her.

What this guide is, and is not

IS

- A latch checklist with line drawings
- A library of feeding positions and what each one is good for
- A decision tree for the most common what-feels-off scenarios
- A reality page about milk supply
- A pumping companion
- A combo-feeding page (no shame)
- A gentle-weaning path
- A page to bring to a lactation consultant

IS NOT

- A diagnosis tool. We do not diagnose mastitis, tongue tie, thrush, oversupply, or anything else.
- A replacement for an IBCLC, a pediatrician, or an OB.
- A breastfeeding-evangelism document. Fed is the requirement. The method is yours.
- Photo-based. The drawings are line drawings, on purpose, so the guide can sit on a counter without anyone looking sideways at you.

The latch checklist

Use this anytime you are not sure if the latch is right. There is room in the printed guide for a small drawing next to each. Read it as a scan, not a quiz. Most latches are fine on 7 of 10. You're looking for the one that is off.

- 1. WIDE MOUTH BEFORE LATCH
The baby's mouth opens wide, not just a little, before coming on. Not a small "o". A big yawn-shaped mouth.
- 2. CHIN INTO BREAST FIRST
Chin contacts the breast before the mouth closes.
This puts the nipple toward the roof of the mouth, where it should be.
- 3. ASYMMETRIC LATCH
More areola visible above the upper lip than below the lower lip. The latch should be lopsided, not centered on the nipple.
- 4. LOWER LIP FLANGED OUT
Like a fish. Curled outward, not tucked in.
If you can't see it, gently pull the lower lip down to check.
- 5. NOSE FREE
The baby can breathe. You don't need to press the breast away from the nose. A correctly-latched baby tilts the head back slightly and the nose is fine.
- 6. CHEEKS ROUNDED, NOT DIMPLED
Dimpling cheeks usually means a shallow latch.
- 7. EARS WIGGLING
Subtle, but real. If the ears move slightly when the baby swallows, the jaw is working.
- 8. AUDIBLE SWALLOWS
After your milk comes in (day 3-5), you should hear small "kuh" sounds during active feeding, not gulps.
- 9. NO PAIN AFTER THE FIRST 10 SECONDS
The first few seconds of latch can be tender.
After that, pain is information. It means something is off. It does not mean you are weak.
- 10. RHYTHMIC SUCK-SUCK-SWALLOW

Two to three sucks, then a swallow. Pauses between bursts. The baby comes off when finished, not when given up on.

If three or more of these are missing, that is a sign to reset the latch. Unlatch (slide a clean finger into the corner of the mouth to break suction), reposition, try again.

The position library

Six positions. One page each in the printed guide. Each one is a tool, not a moral commitment. Switch when you need to.

Cross-cradle

Baby across your lap, supported by the opposite arm to the side you are feeding from.

Good for: small babies, the early days, getting visual confirmation of the latch.

Watch for: shoulder pain in the supporting arm. Use pillows.

Cradle hold

Classic position. Baby across your lap, supported by the same arm as the side you are feeding from.

Good for: older babies who latch easily, longer feeds.

Watch for: a baby that has slid down and is now latched poorly. Reset.

Football (clutch)

Baby tucked under your arm, body alongside your side, head at the breast.

Good for: C-section recovery (no weight on incision), small babies, oversupply, twins (one on each side).

Watch for: the baby's head being pushed in by your arm. Loosen.

Side-lying

You on your side, baby on their side, facing you, latched.

Good for: nights, exhausted feeds, C-section recovery once you can roll, comfort.

Watch for: falling asleep with the baby in bed. Set yourself up safely. Move the baby back to the bassinet when finished.

Laid-back (biological nurturing)

You reclined, baby placed tummy-down on your chest. The baby self-attaches.

Good for: re-establishing latch, oversupply, first-time fumbles, late-onset feeding issues.

Watch for: nothing. This position fixes more than it breaks.

Dangle (or pendulum)

You on hands and knees, baby on their back below you, breast hanging down to the baby's mouth.

Good for: plugged ducts. Gravity pulls toward the plug.

Watch for: tired forearms. Use it as a one-feed tool, not a regular position.

The what-feels-off decision tree

The decision tree lives across two pages in the printed guide. Each branch ends in either “try this” or “call”.

Branch 1, pain on latch only

If the first seconds hurt but the rest of the feed doesn't:

→ Reset the latch. Use the checklist above. → Try laid-back position for one feed. → If pain persists for 48 hours, **call an IBCLC**. Tongue tie or lip tie can present this way and you cannot self-diagnose.

Branch 2, pain throughout the feed

If it hurts the whole time, every feed:

→ Reset, then watch position. → Check for a shallow latch (chin too close to chest, nose too pressed to breast). → Check for thrush signs (shiny pink nipples, burning sensation, white patches in baby's mouth). **Call your provider** if you suspect thrush.

Branch 3, pain after the feed

A burning or stabbing sensation between feeds:

→ Suspect vasospasm. Warm, moist heat after feeds. Avoid cold. → Suspect a deep latch issue compressing the nipple. **Call an IBCLC**.

Branch 4, cracks or bleeding

Cracks at the base of the nipple:

→ Reset the latch first. Cracks are a symptom of the latch, not a thing to treat alone. → Apply your own milk after each feed and air-dry. → A small dab of plain lanolin or hydrogel pads. → **Call an IBCLC** if cracks have not improved in 48 hours. → **Call your provider** if cracks become red, hot, or develop pus. That is an infection.

Branch 5, a hard spot in the breast

A localized hard, tender area:

→ Continue feeding. Position the baby's chin pointed at the hard spot. → Warm compress before feeding, cool after. → Try dangle feeding for one feed. → **Call your provider** if you develop a fever, body aches, or the area is red and hot. That is mastitis. It needs antibiotics, often urgently.

Branch 6, the milk doesn't seem to be enough

Suspecting low supply:

→ Check the diaper output. Six to eight wet diapers by day 5, regular dirty diapers, weight gain at appointments. If those are happening, supply is okay. → Pumping output is not the same as breastfeeding output. Babies are more efficient than pumps. → Cluster feeding, especially evenings, is normal. It is not a low-supply sign. → **Call an IBCLC** if diaper output is low or the baby is losing weight.

Branch 7, the milk seems too much

Suspecting oversupply:

→ Try laid-back position. Gravity helps. → Try block feeding (one side per session, for 2-3 sessions, then switch). → **Call an IBCLC** if the baby is choking at feeds, has very green watery stools, or is gassy and unsettled.

A milk-supply reality page

What actually affects milk supply, in plain language. What looks like a supply problem but isn't. What you can ignore.

What actually moves supply

- **Frequency.** Removing milk often signals the body to make more. Less frequent feeding = less milk. This is the main lever.
- **Effective removal.** A baby with a good latch removes milk well. A pump set up incorrectly does not. Address the latch or the flange size before assuming low supply.
- **Sleep.** Severe sleep deprivation lowers supply over time.
- **Hydration.** Drinking water near baseline matters. Mega-hydrating does not increase supply beyond baseline.
- **Stress, severe.** Acute, prolonged stress (not normal new-parent stress) can affect supply.
- **Certain medications.** Some birth control, some cold meds (especially with pseudoephedrine), some others. Check with your provider when starting any new med.

What does NOT meaningfully affect supply

- Eating one specific food
- Drinking exactly X ounces of water
- Whether the breasts feel "full" or "soft" (after the first 6 weeks, breasts soften even with good supply)
- A pump output that seems low (pumps are not babies)
- The shape, size, or appearance of your breast or nipple

When to actually worry

If the baby:

- Is not gaining at appointments
- Has fewer than 6 wet diapers a day after day 5
- Is hard to wake for feeds
- Seems unsatisfied for many hours after every feed
- Has a dry mouth or sunken soft spot

Any of those, **call your pediatrician today.**

The pumping companion

Two pages in the printed guide.

A flange size cue

If the nipple is rubbing against the side of the flange tunnel, the flange is too small. If the areola is being pulled into the tunnel with each cycle, it's too big.

Wallet-card guidance: measure the nipple at rest, in millimeters. Add 2-3mm for a starting flange size. Then adjust.

A pumping schedule

If you are exclusively pumping or pumping to maintain supply while at work:

- Newborn: every 2-3 hours, including overnight, ~8-12 times in 24 hours
- 3 months: every 3 hours during the day, longer stretch overnight
- 6 months: every 3-4 hours, supply usually stabilized

Output expectations (rough)

- Newborn: a few drops to half an ounce per session
- Days 5-10: 1-2 oz per session
- Steady state (3+ months): 3-4 oz per session, often more from the first morning pump

Your steady state may be much higher or lower. The number to optimize is "is the baby growing." Not "what does the bottle look like."

Combo feeding

A page for the question that is treated like a confession.

There is no rule that says feeding must be exclusively breast or exclusively bottle. Many families do a combination, by choice or by circumstance. Babies are fed. That is the goal.

Some things to know about combo feeding:

- Adding formula reduces breastfeeding demand, which can reduce supply over time. If you want to keep milk supply up, replace a breastfeed with a pump session when offering a bottle.
- Bottle latch is different from breast latch. Use a slow-flow nipple and pace the feeding (hold the bottle horizontal, give pauses). This is called paced bottle feeding.
- Some babies develop a preference. Most do not.
- Mixing formula and breast milk is fine, but use the breast milk first and the formula second, so you don't waste pumped milk.

If you are adding formula because you have to (work, supply, mental health, choice), do it without apology. The version of you who is fed and able to function is the version your baby needs.

Weaning gentle

A two-page reference for whenever you decide. The decision is yours. It can be at week six, six months, or three years.

General principles

- Drop one feed at a time. Wait 3-5 days before dropping another.
- Drop the least-favorite feed first (often midday).
- Save the most-important feed (often bedtime) for last.
- Engorgement is common during weaning. Cold compresses, comfort nursing only as needed, ibuprofen if your provider okays it.

A weaning order, rough

1. Replace one daytime feed with a bottle or solid (depending on age)
2. Wait 3-5 days
3. Replace another
4. Continue
5. Last: bedtime feed
6. Then: night feeds, if still happening

A note

Weaning can be emotional. Many parents feel a wave of low mood ("weaning sadness") that is a real hormone shift, not a personality failure. If the low mood persists more than two weeks, **call your provider**.

A page to bring to the IBCLC

Tear this out (or photograph it) before your appointment.

DATE: _____ BABY'S AGE: _____

WHAT'S HAPPENING

The thing that is hardest right now (one sentence):

WHAT I'VE NOTICED

Pain (circle): none on latch throughout after

Pain location: _____

Pain description: sharp / burning / aching / stabbing
 other: _____

The latch (which of these I see):

- ☐ Wide mouth ☐ Lower lip flanged out
- ☐ Asymmetric ☐ Audible swallows
- ☐ Rounded cheeks ☐ Comfortable after 10 seconds
- ☐ Not sure

THE BABY

Wet diapers per 24h: _____

Dirty diapers per 24h: _____

Feeds per 24h: _____

Average feed length: _____ minutes

Last weight: _____ (date: _____)

Weight gain trend: ☐ steady ☐ slow ☐ losing

WHAT I'VE TRIED

- ☐ Reset the latch
- ☐ Different positions (which: _____)
- ☐ Lanolin / hydrogel
- ☐ Pumping (output: _____ oz, frequency: _____)
- ☐ Block feeding
- ☐ Laid-back position
- ☐ Other: _____

WHAT I WANT FROM THIS APPOINTMENT

- ☐ A look at the latch
- ☐ Help with positions
- ☐ A plan if combo-feeding
- ☐ A referral for tongue tie evaluation
- ☐ A plan to wean
- ☐ Just to be told it's normal
- ☐ Other: _____

QUESTIONS I WROTE DOWN

1. _____
2. _____
3. _____

Numbers to keep

LACTATION CONSULTANT (IBCLC)

Name: _____ Number: _____

PEDIATRICIAN

Name: _____ Number: _____

After-hours line: _____

OB / MIDWIFE

Name: _____ Number: _____

US NATIONAL BREASTFEEDING HELPLINE

1-800-994-9662 (Mon-Fri, English + Spanish)

LA LECHE LEAGUE LOCAL

Local chapter: _____

US MATERNAL MENTAL HEALTH HOTLINE

1-833-852-6262 (24/7, free, confidential)

US SUICIDE & CRISIS LIFELINE

988 (call or text, 24/7)



From Soothemade Notes, a small apothecary of printables, planners, and cards for the unphotographed parts of new parenthood. Made slowly, in plain language.

Not medical advice.



Soothemade *Notes*

*Made for the version of you no one tells you about, the
one at four in the morning, the one in the parking lot, the
one who doesn't recognize her own week.*

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